# Copilot Efficiency Plan

*Regional Outpatient Clinic | Prepared by: Health Administration Coordinator | October 27, 2026*

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## Executive Summary

Prioritize clinician-reviewed Dragon Copilot encounter-note drafting and staff-reviewed Microsoft 365 Copilot portal-response drafting. Together they account for 43.0 of the clinic's 76.5 documented weekly hours lost across six audited activities; the pilot will measure actual time saved, quality, and safe handoff performance rather than assume all baseline loss is recoverable.

## Section 1 — Automation Suitability Summary

Of four tasks evaluated, one is a strong candidate, two are conditional candidates, and one is not suitable for autonomous Copilot completion.

| Task | Repeatability | Risk | Review need | Rating | Evidence-based rationale |
|---|---|---|---|---|---|
| **1. Pre-visit reminders and preparation messaging** | High | Low | Minimal | Strong candidate | ~120 routine messages/week use copied templates; staff verify appointment time and fasting/preparation flags. A wrong prep flag can reschedule a lab draw. |
| **2. Draft encounter SOAP note (Dragon Copilot)** | High | Moderate–high | High / mandatory | Conditional candidate | ~150 encounters/week; manual notes take 8–12 minutes. Audio-to-SOAP drafting is repeatable, but missed symptoms, dosage errors, or wrong history require clinician editing and EMR sign-off. |
| **3. Initial prior-authorization submission decisions** | Medium | High | High | Not suitable | ~25 complex medication requests/week involve payer policy, medical necessity, and appeals. Incorrect clinical judgment can cause denial, delayed care, and financial liability; no autonomous final decision. |
| **4. Daily portal inbox triage and** | High | Low–moderate | Low–moderate | Conditional candidate | ~200 mixed refill, scheduling, billing, |

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| **tagging** | | | | and urgent clinical messages/week. Suggested tags aid sorting but staff must detect urgent symptoms before routing. |

These ratings distinguish assistance from autonomous action. Task 1 permits near-template generation with a short pre-send check. Tasks 2 and 4 need human-in-the-loop safeguards because clinical content or urgency can be misread. Task 3 may receive clerical information-gathering support, but submission or appeal decisions remain with authorized personnel.

## Section 2 — Time-Loss Analysis and Opportunity Inventory

The priorities are post-visit encounter-note synthesis (documentation) and individualized routine portal-response drafting (communication), the two largest directly addressable time-loss patterns.

| Category | Observed inefficiency | Lost time (clinic total) | Copilot addressability | Recommended tool / feature |
|---|---|---|---|---|
| **Documentation** | Manual post-visit note synthesis, SOAP reformatting, EMR history re-entry | 24.5 hr/week | Direct | Dragon Copilot ambient clinical capture and SOAP draft generation |
| **Documentation** | Compile past specialist notes from fragmented PDFs and chart entries | 8.0 hr/week | Partial | M365 Copilot in Word/Teams: document summarization; chart cross-check |
| **Communication** | Write individualized routine portal answers from scratch | 18.5 hr/week | Direct | M365 Copilot in Outlook: draft with Copilot and tone adjustment |
| **Communication** | Telephone missed-visit follow-up and confirmation calls | 6.5 hr/week | Indirect | M365 Copilot in Outlook: batch follow-up scheduling preparation; calls still needed |
| **Clerical** | Read/tag daily inbox and route messages into specialty folders | 12.0 hr/week | Partial | M365 Copilot in Outlook/Teams: summaries and suggested tags with human triage |
| **Clerical** | Re-key demographics/insurance from fax referrals into EMR | 7.0 hr/week | Partial | M365 Copilot: extract proposed form fields; verify against fax and EMR |

## Prioritization Rationale

The four-week, ten-staff audit identifies 76.5 clinic hours lost weekly across the six activities. Encounter-note synthesis is first at 24.5 hours per week and has direct addressability: Dragon Copilot can convert encounter audio into a draft SOAP note, eliminating substantial retyping and format assembly while preserving mandatory clinician judgment. Routine portal-response drafting is second at 18.5 hours per week and also directly addressable: Outlook Copilot can create consistent, editable responses from approved details. Their combined 43.0 hours represent approximately 56% of measured weekly loss, although actual savings must be established in a pilot.

The next largest pattern, clerical inbox triage, loses 12.0 hours weekly but is only partially addressable because urgent clinical language and correct queue assignment require manual verification. Specialist-history compilation (8.0 hours) and fax abstraction (7.0 hours) also require source reconciliation; missed-appointment calling (6.5 hours) is only

indirectly aided by draft follow-up organization. Therefore, the selected priorities have both the highest baseline burdens and the clearest draft-generating capabilities. Team leads should track net handling time, correction rates, routing/safety incidents, and clinician approvals weekly. No automated draft is equivalent to final clinical authorization or a verified patient response.

## Section 3 — Workflow Refinement Specifications

### Specification 1: Post-Visit Encounter Note Drafting

Target tool: Microsoft Dragon Copilot — ambient clinical capture, structured SOAP draft; final reconciliation within the EMR.

Standardized Copilot prompt (scope, context, format, review): “Create a DRAFT SOAP note for this completed encounter only. Use the authorized ambient encounter transcript and clinician-provided visit notes; do not infer missing symptoms, findings, diagnoses, or orders. Format as Subjective, Objective, Assessment, and Plan, with clearly marked [VERIFY] items for ambiguous names, dosages, negatives, and follow-up instructions. Flag contradictions or missing evidence. The treating clinician must compare every section with the encounter and approved chart data, correct errors, and sign in the EMR; do not finalize or sign.”

Review checklist (Task 2: conditional; moderate–high risk; mandatory clinician review): (1) Match patient/visit, chief complaint, history, symptoms, examination and negative findings to actual encounter. (2) Check medication names, doses, allergies, diagnosis, orders, and follow-up against clinician intent; resolve every [VERIFY] item. (3) Confirm SOAP placement, minimal necessary access, chart destination, and no unverified or unintended content; clinician alone approves/signs.

Handoff timing: Immediately after the clinician closes the encounter/capture, send the draft to that clinician’s EMR review queue; sign-off follows reconciliation, before the note is marked complete.

Measurable target: Reduce median manual note preparation from the scenario’s approximately 10–12 minutes to under 4 minutes per encounter, while achieving at least 95% clinician sign-off within 4 hours and 100% clinician verification before finalization. Audit timestamps and corrections weekly.

### Specification 2: Routine Patient Portal Response Drafting

Target tool: Microsoft 365 Copilot in Outlook/approved portal workflow — draft with Copilot and tone adjustment.

Standardized Copilot prompt (scope, context, format, review): “Draft one administrative reply to this routine patient portal message about appointment preparation, appointment status, or documented refill status. Use only the supplied message and coordinator-verified scheduling/status facts in the approved clinic workspace; do not access or invent clinical guidance. Provide a courteous plain-language greeting, direct answer, next step, and closing in no more than 120 words. Mark missing details [CHECK WITH STAFF]. Before sending, the coordinator must verify patient/recipient, factual accuracy, authorized channel, and whether symptoms or medication concerns require clinical escalation.”

Review checklist (Task 1-style standardized communication, but clinical messages carry Task 4 conditional triage risk): (1) Confirm identity, portal address, appointment/status facts, and preparation instructions against source records. (2) Screen for urgent symptoms, adverse reactions, or clinical questions; do not answer those administratively—route immediately to clinical staff under clinic protocol. (3) Remove unsupported content and unnecessary PHI, check empathy/accessibility, then staff member approves the send.

Handoff timing: When a coordinator opens an unassigned routine portal ticket and verifies its category, generate a draft for coordinator review; route suspected urgent clinical content to the clinical queue immediately rather than drafting a routine answer.

Measurable target: Reduce median routine-response handling time from the scenario’s 6–8 minutes (7-minute midpoint) to under 2.5 minutes, with 100% staff review before sending and zero known urgent messages sent as routine replies. Measure ticket timestamps and reviewed samples weekly.